

BCS Foundation Certificate in Business Analysis

Body of Proof Portfolio | Sprint 1: HealthTech | MediCore AI

Week 2 Deliverable: Strategic Analysis Report

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Deliverable Purpose

This deliverable forms part of the BCS Foundation Certificate in Business Analysis Body of Proof Portfolio. It presents a comprehensive strategic analysis of MediCore AI, a fictional HealthTech startup preparing to launch an AI-assisted triage product within the NHS. The analysis applies three recognised BA strategy techniques - PESTLE, VMOST, and SWOT - to contextualise the external environment, validate strategic coherence, and assess internal capability against external risk.

Section 1: PESTLE Analysis

Technique Rationale

PESTLE was selected as the primary external environment scanning tool because MediCore AI operates at the intersection of two highly regulated and rapidly evolving domains: NHS healthcare and artificial intelligence. The technique systematically surfaces factors across six dimensions that could materially affect strategic viability. For a Business Analyst, PESTLE provides the evidential baseline from which scope, risk, and strategic alignment can be challenged - ensuring that business decisions are grounded in structured environmental evidence rather than assumption.

PESTLE Table - MediCore AI

Factor	Key Environmental Factors	Implication for MediCore AI
Political	UK Government Pro-Innovation AI Regulation white paper; NHS Long Term Plan (2019); NHSE AI Lab actively funding and validating health AI products across Trusts	MediCore AI operates within a government-endorsed policy environment. This means that NHSE validation pathways provide a credible and structured route to market adoption - MediCore must align its go-to-market strategy with NHSE approval mechanisms.
Economic	Constrained NHS procurement budgets; high upfront implementation costs; projected long-term savings through reduced A&E escalations and improved patient throughput	MediCore must present a compelling ROI narrative. This means that financial modelling - including payback period and projected cost savings per Trust - is a prerequisite to any ICB engagement.
Social	Public distrust of AI in clinical decision-making; patient safety concerns; risk of resistance from clinical staff unfamiliar with AI-assisted workflows	MediCore must invest in clinical trust-building and transparent communication of AI accuracy benchmarks. This means that a structured change management plan targeting clinical staff is required before deployment - not after.
Technological	Fragmented NHS legacy IT infrastructure across Trusts; HL7 FHIR interoperability standards; NHS Federated Data Platform providing centralised, cross-Trust data infrastructure	MediCore must adopt an API-led integration strategy aligned to HL7 FHIR standards. This means that the NHS Federated Data Platform can substantially reduce proprietary data infrastructure costs by leveraging existing NHS investment.
Legal	ISO 42001 AI Management Systems standard; GDPR and Data Protection Act 2018; NHS Data Security and Protection (DSP) Toolkit; MHRA medical	Compliance must be embedded in the solution architecture from inception. This means MediCore cannot commence deployment without satisfying all four regulatory frameworks - a compliance-by-design approach is the only viable path.

	device classification for AI diagnostic tools	
Environmental	NHS Net Zero 2040 target (Greener NHS programme); AI model training energy consumption; sustainable procurement criteria applied to NHS supplier selection	MediCore must demonstrate sustainable operational practices. This means that energy-efficient infrastructure and transparent carbon reporting will be required as NHS sustainable procurement criteria continue to mature.

Section 2: VMOST Analysis

Technique Rationale

VMOST was selected to establish and validate the vertical strategic thread connecting MediCore AI's long-term vision to its immediate operational actions. As a BA technique, VMOST is particularly valuable in change contexts because it enables scope challenge at every level: any requirement or initiative that cannot be traced to a stated objective, strategy, or mission should be questioned. VMOST also provides the governance framework that connects the PESTLE environmental analysis to internal strategic decision-making - ensuring that strategic choices are coherent with the external environment being entered.

VMOST Table - MediCore AI

Level	Statement and Detail
Vision	A UK healthcare system where AI-assisted triage eliminates preventable harm, reduces patient wait times, and frees clinical professionals to deliver personalised, human-centred care.
Mission	To optimise NHS patient flow and clinical decision-making by embedding AI-driven triage capabilities into existing healthcare infrastructure - delivering faster access for patients and better-informed prioritisation for clinicians.
Objectives	• O1: Reduce average triage-to-clinician wait time by 30% across pilot NHS Trusts within 18 months of deployment • O2: Achieve 80% clinical staff adoption of the AI triage tool across all pilot sites within 12 months of go-live • O3: Deliver a measurable 15% reduction in avoidable clinical escalations per Trust within 18 months of deployment • O4: Maintain an AI triage accuracy rate of 95% or above against clinician benchmark decisions throughout the full pilot period
Strategy	• S1: Adopt an API-led integration strategy leveraging existing NHS-approved AI tools - minimising capital expenditure and accelerating time to deployment • S2: Enter the market via a controlled pilot with a single Integrated Care Board (ICB) to generate a representative, validated dataset before scaling nationally • S3: Embed regulatory compliance by design - incorporating ISO 42001, GDPR, MHRA requirements, and NHS DSP Toolkit obligations into the solution architecture from inception
Tactics	• T1: Conduct a technical landscape audit of existing NHS-approved AI tools and triage workflows to identify high-impact integration touchpoints - outputs: integration map and recommended API connections • T2: Develop and present a formal business case to a target ICB - quantifying the cost of non-implementation and projecting post-deployment outcomes - outputs: business case document and sign-off meeting • T3: Commission a regulatory gap analysis comparing the proposed solution against ISO 42001, GDPR, MHRA, and DSP Toolkit requirements - outputs: gap register and remediation plan with assigned owners

Section 3: SWOT Analysis

Technique Rationale

SWOT was selected to synthesise the outputs of the PESTLE and VMOST analyses into a single, decision-support framework. Where PESTLE maps the external environment and VMOST establishes strategic intent, SWOT bridges them by assessing whether MediCore AI has the internal capability to execute its stated strategy within the identified external context. For a BA, SWOT is most valuable not as a standalone tool but as a synthesis mechanism - it reveals misalignments between ambition and capability, and between opportunity and threat. The four quadrants, read in cross-combination, directly inform risk management and change prioritisation decisions.

SWOT Matrix - MediCore AI

STRENGTHS Internal, Positive	WEAKNESSES Internal, Negative
- Existing API access and technical infrastructure reduces time to deployment - no ground-up build required - Strong problem-solution fit: the product directly addresses an evidenced NHS operational pain point in patient flow, wait times, and avoidable clinical escalations	- Technical implementation knowledge gap - specialist AI/ML engineering capability not yet confirmed in-house - Staff competency gap - clinical and operational staff require structured upskilling before the tool can be effectively adopted at scale - No Clinical Digital Safety Officer (CDSO) with AI competency confirmed - represents a regulatory and governance risk prior to deployment
OPPORTUNITIES External, Positive	THREATS External, Negative
- NHS Federated Data Platform provides a scalable, trusted data foundation for AI model training and real-time clinical data integration across Trusts - NHSE policy tailwind - active government promotion of AI adoption creates a receptive commissioning environment and reduces regulatory resistance - First-mover advantage in NHS AI triage - limited direct competitors currently operating at scale in this specific clinical domain	- ISO 42001 and MHRA medical device regulatory complexity - compliance is non-negotiable but resource-intensive and time-consuming - Public and clinical distrust of AI in healthcare - adoption risk if trust-building is not embedded from the outset of deployment strategy - GDPR and NHS DSP Toolkit constraints limit how patient data can be stored, processed, and shared across Trust boundaries - NHS budget pressure - procurement decisions are slow and cost-sensitive, extending sales cycles and creating funding uncertainty

Strategic Synthesis

Cross-analysis of the four quadrants reveals three key strategic insights for MediCore AI:

- Strengths vs Opportunities: MediCore's existing API infrastructure and strong problem-solution fit directly enable exploitation of the NHS Federated Data Platform and NHSE policy

tailwind - reinforcing the API-led integration strategy (S1) and accelerating ICB engagement (S2).

- Weaknesses vs Threats: The staff competency gap and absence of a Clinical Digital Safety Officer amplify the ISO 42001 and MHRA regulatory complexity threat. Appointing a CDSO and commissioning the regulatory gap analysis (T3) must be treated as Day 1 priorities - not deferred to the post-pilot phase.
- Strengths vs Threats: Strong problem-solution fit and evidenced clinical outcomes from the ICB pilot will be the most effective counterweight to public and clinical scepticism about AI in healthcare - directly addressing the trust deficit identified in the Social (PESTLE) and Threats (SWOT) quadrants.